

Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°133/MVE-BDBV/24/09/2026

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HIGHLIGHTS (last 24 hours)

- Fifty-six (56) new confirmed cases of Ebola Virus Disease (EVD), including 23 community deaths, have been reported. The confirmed cases reported in the last 24 hours were distributed across the provinces of Ituri (35 cases), from North Kivu (17 cases), Haut-Uélé (3 cases) and Bas-Uélé (1 case). Eleven (11) deaths were recorded in the CTEs.
- Of the 446 new samples analyzed, 56 were found to be positive for the Bundibugyo virus, representing a positivity rate of 12.6%.
- No new health zones have been affected in the last 24 hours.

ACCUMULATIVE CAS 7 946	ACCUMULATIVE DEATH CONFIRMED 3 833	LETHALITY 48,2 %	PATIENTS IN ISOLATION/CTE 848	ACCUMULATIVE HEALED 2 001	TRACKING RATES CONTACTS (Of the Day) 79,2 %
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Affected provinces and affected health zones



7 Provinces affected

Uele, Ituri, North Kivu,
South Kivu, Tshopo, Lower Uele and
Sud Ubangi



63 out of 167
health zones affected
(37,7%)

Summary of key points from the last 24 hours

In the past 24 hours, **56 new confirmed cases** of Ebola Virus Disease (EVD) including 23 community deaths have been recorded: 35 cases in Ituri, 17 in North Kivu, 3 in Haut-Uélé and 1 in Bas-Uélé.

Eleven (11) deaths were recorded in the CTEs (5 in Ituri, 4 in North Kivu, 1 in Haut-Uélé and 1 in Tshopo). Thus, a total of **34 confirmed deaths** were recorded for September 24, 2026.

In total, **35 patients** (27 in Ituri, 6 in North Kivu, 1 in Haut-Uélé and 1 in Tshopo) were declared cured after obtaining two negative results for Ebola virus disease.

A total of **7,946 confirmed cases, including 3,833 deaths**, had been reported as of September 24, 2026, representing a case fatality rate of **48.2 %**. Ituri province remains the epicenter, accounting for **76.4%** of reported confirmed cases and **62.5%** of new confirmed cases.

No new health zones have been affected in the last 24 hours.

hours. Thus, **63 out of 167 health zones (37.7%)** have been affected in **7 provinces** since the start of the epidemic. Ituri has 28 out of 36 affected health zones, followed by North Kivu (16/34), Haut-Uélé (7/13), Tshopo (7/23), Bas-Uélé (3/11), South Kivu (1/34) and South Ubangi (1/16).

Table 1. Distribution of confirmed cases and deaths by affected province, as of September 24, 2026.

Province	New confirmed cases (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	35	6 067	2 784	45,9%	28/36 (77,8 %)
North Kivu	17	1 497	896	59,9%	16/34 (47,1 %)
Haut-Uélé	3	325	131	40,3%	7/13 (53,8 %)
Boss	0	43	16	37,2%	7/23 (30,4 %)
South Kivu	0	3	1	33,3%	1/34 (2,9 %)
Bas-Uélé	1	9	4	44,4%	3/11 (27,3%)
Sud Ubangi	0	2	1	50,0%	1/16 (6,3%)
Total	56	7 946	3 833	48,2%	63/167 (37,7 %)

Table 2. Distribution of confirmed cases and deaths by province and zone as of September 24, 2026

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Cas (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	6 067	2 784	45,9%	35	15	5	20
Give	11	1	9,1%				0
Unimond	8	6	75,0%				0
Ariwara	8	2	25,0%				0
Aungba	12	5	41,7%				0
Bamboo	197	42	21,3%				0
have	2	2	100,0%				0
Bunia	1692	535	31,6%	9	3	3	6
Ladies	40	9	22,5%				0
Drodro	15	7	46,7%				0
Fireworks	81	30	37,0%				0
Gethy	7	3	42,9%				0
Flounder	2	0	0,0%				0
Kilo	34	12	35,3%				0
Team	162	96	59,3%	3	2	0	2
Liter	280	140	50,0%	3	1	0	1
Logo	13	5	38,5%				0
Fight	33	8	24,2%	1	0	0	0
distributed	4	1	25,0%				0
Washing	25	9	36,0%	1	0	0	0
Darkness	84	45	53,6%	9	9	0	9
Mangala	342	153	44,7%	1	0	0	0
Mongbwalu	696	325	46,7%	2	0	1	1
Nanny	275	152	55,3%	1	0	0	0
Nizi	763	276	36,2%	1	0	0	0
Legumes	127	37	29,1%				0
Jungle	12	4	33,3%				0

Rwampara	1066	361	33,9%	4	0	1	1
Tchomia	76	25	32,9%				0
A valves*	THAT	493	THAT				0
North Kivu	1 497	896	59,9%	17	8	4	12
Me	318	213	67,0%	12	7	0	7
Farm	18	7	38,9%				0
Butembo	254	163	64,2%				0
Ten	1	0	0,0%				0
Kalunguta	50	16	32,0%				0
Cut	551	352	63,9%	1	0	3	3
Source	1	0	0,0%				0
Kyondo	33	20	60,6%				0
Lubero	21	11	52,4%				0
Be careful.	25	11	44,0%				0
Manguredjipa	13	5	38,5%	0	0	1	1
Masereka	28	9	32,1%				0
The museums	121	65	53,7%				0
Mutwanga	11	2	18,2%				0
Oicha	23	14	60,9%	4	1	0	1
Vuh	29	8	27,6%				0
Haut-Uélé	325	131	40,3%	3	0	1	1
Kill Mangbetu	39	16	41,0%	0	0	1	1
Dumb	1	1	100,0%				0
Gombari	3	1	33,3%				0
Mathematics	100	43	43,0%				0
Lining	74	33	44,6%	3	0	0	0
Beans	2	1	50,0%				0
Ordinary	106	36	34,0%				0
Boss	43	16	37,2%	0	0	1	1
Bafwasende	7	1	14,3%				0
Kabondo	5	1	20,0%				0
Lubunga	1	0	0,0%				0
Maquis-Kisangani	23	10	43,5%	0	0	1	1
Mangobo	5	3	60,0%				0
Boss	1	0	0,0%				0
Wanie-Rukula	1	1	100,0%				0
South Kivu	3	1	33,3%	0	0	0	0
Mili-Murhesa	3	1	33,3%				0
Bas Uélé	9	4	44,4%	1	0	0	0
Blind	1	1	100,0%				0
Viadana	5	1	20,0%	1	0	0	0
Walk	3	2	66,7%				0
Sud Ubangi	2	1	50,0%	0	0	0	0
Hair	2	1	50,0%				0
Total	7 946	3 833	48,2%	56	23	11	34

*These are the confirmed deaths that occurred in the various CTEs in the province which are being broken down to be classified by health zone.

NA : Not Applicable.

Table 3. Status of alerts notified by province for September 24, 2026

DPS	Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CTE/CT (of the day)
	Living	Deceased		Living	Deceased				
Haut-Uélé	28	4	32	7	3	20	0	10	5
Bas-Uélé	73	3	76	9	2	64	0	11	0
Ituri	1 009	62	1071	109	62	900	0	161	91
North Kivu	519	24	543	75	24	421	0	99	75
Sud Ubangi	14	0	14	1	0	13	0	1	0
Boss	141	0	141	8	0	133	0	8	4
Total General	1 784	93	1 877	209	91	1 551	0	290	175

Figure 1. Evolution of cases by date of symptom onset over the last 21 days

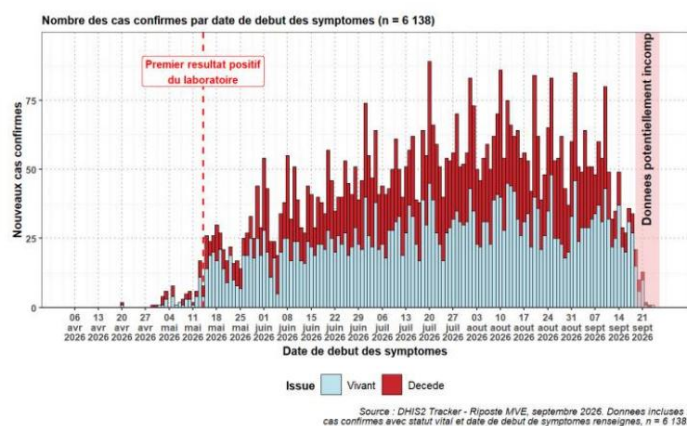
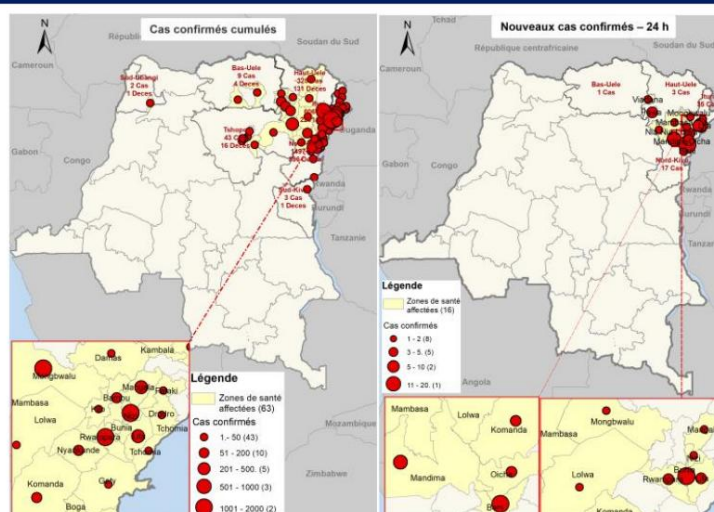


Figure 2. Spatial distribution of confirmed cases (24 hours and cumulative) by health zone.



1. COORDINATION

1.1. Main Actions

- In South Ubangi, a preliminary meeting was held with the MSF team to harmonize the interventions to be implemented in the affected health zone (Bulu).

2. RESPONSE ACTIONS BY PILLAR

2.1. Epidemiological surveillance

2.1.1. Main Actions

A total of **1,877 alerts** were received, of which 1,851 (**98.6%**) were verified. Among these, **300 alerts** were validated as suspected cases and **290 (96.7%)** were investigated, of which **175** were transferred to CT/CTE (Table 3).

Of the **29,542** contacts currently being monitored, **23,392** were seen in the last 24 hours, representing a follow-up rate of **79.2%**, lower than the previous day's rate of 83.4%. In South Ubangi, this rate was 94.9% (93/98), 85.9% (116/135) in Bas-Uélé, 84.1% (11,682/13,884) in North Kivu, 76.7% (10,405/13,558) in Ituri, and 75.7% (827/13,884) in South Ubangi. 093) in Haut-Uélé, 34.8% (269/774) in Tshopo.

2.2. Update on checkpoint and point of entry (PoE/PoC) activities

2.2.1. Main actions

During the day of September 24, 2026, 9 alerts were notified to the PoE/PoC, including 4 in Ituri, 5 in North Kivu: Two lifeless bodies were intercepted at the PoC of Mudzibala (ZS of Bunia) and of Sans Plainte (ZS of Ariwara) without being swabed, and two live alerts were validated, one transferred to the CTE of Bunia and the other escaped before its transfer from the Poipo PoC (ZS of Mongbwalu); Two lifeless bodies were intercepted at the Mavivi Barrière PoC and directed to the morgue for swabbing; one live alert was validated at the Maboya PoC and two others invalidated at the Kasitu PoC.

Table 4. Status of key PoE/PoC indicators by province, as of September 24, 2026

Indicators	Ituri	North Kivu	Boss	Global
Report completeness	98,3% (59/60) 100%	(21/21) 100% (18/18)	99,0% (98/99)	
% of functional strategic PoE/PoC	100%	100%	100%	100%
Number of people who have passed PoE/PoC	141 449	92 677	21 114	255 240
% of travelers screened	97,3%	97,5%	99,1%	97,5%
% of travelers who washed their hands % of	96,9%	97,5%	99,1%	97,3%
travelers made aware	99,4%	97,5%	100,0%	98,8%
Number of alerts notified	4	5	0	9
Number of validated live alerts (suspected cases)	2	1	0	3
Number of bodies intercepted today % of suspected	2	2	0	4
cases transferred to CTE/CT	50,0%	100,0%	THAT	66,7%
% of bodies swabs/secured % of suspected cases	0,0% (0/2)	100,0% (2/2)	THAT	50,0% (2/4)
investigated within 4 hours	50,0%	100,0%	THAT	66,7%

2.2.2. Challenges

Two bodies were intercepted in Ituri (Mudzibala and Sans Plainte) and not yet identified, and a suspect escaped from the Poipo PoC; in Ituri, the Avakubi PoC is not operational due to insecurity, resulting in a low seizure rate. alerts in DHIS2 Tracker, insufficient mobility in the health zones of Rwampara, Nizi and Bunia and delayed supply of inputs to some PoCs; in North Kivu, persistent threats against the providers of the PoCs on the Butembo axis and delayed handling of alerts by the alert cells.

2.3. Laboratory

2.3.1. Main actions

- **Ituri: 35 new positive results** (20 living and 15 deaths) out of 202 new samples received and analyzed (**positivity: 17.3%**).
- **North Kivu: 17 new positive results** (9 living and 8 dead) out of 218 samples received and analyzed (**positivity: 7.8%**).
- **Haut-Uélé: 3 new positive results** (3 living and 0 deaths) out of 12 samples received and analyzed (**positivity: 25.0%**).
- **Tshopo: 8 samples received and analyzed (8 live)** all tested negative.
- **Bas-Uélé: 1 new positive result (1 live sample)** out of 5 samples received and analyzed (**positivity: 20.0%**).
- **South Ubangi: 1 sample** received is currently being analyzed.

2.4. Infection Prevention and Control (IPC/EDS)

2.4.1. Main actions

- **In Ituri**, 16 out of 27 rings were opened (59.3%), and all 47 identified households were decontaminated (100%); 10 out of 11 health facilities (90.9%) and 2 schools were decontaminated, without the distribution of hygiene kits to households or infection prevention and control (IPC) kits to health establishments. Regarding blood and health surveillance (BHS), 72 alerts were recorded (including 11 postponements from the previous day), 54 BHS were carried out (75.0%), and 52 bodies were swabbed; 9 swabbing failures were reported.
- **In Haut-Uélé**, 2 out of 6 treatment areas were opened (33.3%), 19 out of 30 households were decontaminated (63.3%), and both of the affected health establishments (ESS) were decontaminated (2/2); 6 out of 10 ESSs were visited, 2 out of 7 received infection prevention and control (IPC) kits, and 23 out of 28 service providers (82.1%) were briefed. Regarding the health monitoring centers (EDS), 3 alerts were received (Isiro, Wamba, and Pawa), 3 bodies were swabbed, and 3 EDS were carried out.
- **In Bas-Uélé**, the two expected infection control zones were opened in Viadana (100%), and one out of two households was decontaminated (50.0%). Eleven health zones (ESS) in Buta, Ganga, and Viadana were monitored, with 31 out of 33 service providers (93.9%) and 11 out of 13 hygiene workers (84.6%) briefed. Nine of the 13 isolation providers at Buta General Referral Hospital (69.2%) received reinforcement. Regarding the blood and casualty incidents (BSIs), three alerts were received (Viadana, Ganga, and Poko), two bodies were swabbed, no BSIs were carried out, and one failure was reported in Poko. No data was transmitted by Tshopo.
- **In South Ubangi**, two health facilities in the Gwaka health district are under infection prevention and control (IPC) surveillance with a high risk of exposure: the Gwaka health center, frequented by a suspected case and the confirmed case, underwent an IPC assessment and preventive decontamination of the facility and 10 households, and two wards of the Miluna hospital were preventively decontaminated; the local health center (ECZ) has been trained in IPC. As for the local health units (EDS), no alerts have been recorded and the EDS team is currently being pre-positioned.

2.4.2. Challenges

In Ituri: ring road openings limited to 59.3% (16/27) with gaps in Komanda, Mangala, and Mandima; 9 swab failures due to a lack of supplies and teams in Mandima and family refusals; insufficient ECUMER coverage, lack of EDS supplies, resistance in Nyankunde, and insufficient transportation in Kilo, Nia-Nia, Mandima, Damas, and Fataki. In North Kivu: lack of reporting of IPC activities around cases and insecurity for EDS teams in Beni. In Haut-Uélé: low ring road openings (33.3%; 2/6), refusal by 2 households in Wamba, and slow data reporting from some health zones. In Bas-Uélé: household decontamination at 50.0% (1/2), lack of WASH kits for households and IPC supervisors.

2.5. Holistic care

2.5.1. Main actions

- **In Ituri**, 72 new admissions were recorded during the day, compared to 117 discharges, including 27 recoveries. Thus, 394 patients are hospitalized out of 1015 beds (occupancy rate of 38.8%); 360 out of 378 patients (95.2%) received hot meals, and children under LNPE/SLM were fully covered at the CTE (15/15), at the nursery (9/9), and in the community (33/33); 39 patients are in critical condition.
- **In North Kivu**, 75 new admissions were recorded compared to 79 discharges, including 6 recoveries; 365 Patients are hospitalized, including 251 in standardized care facilities, representing an occupancy rate of 81.5% (354 beds); 114 patients (31.2%) remain under the care of... Non-standardized centers and 13 patients are in critical condition. The 168 patients, 98 accompanying persons and 328 frontline providers received hot meals.
- **In Haut-Uélé**, 9 new admissions were recorded compared to 5 discharges (1 recovered). Thus, 66 patients are hospitalized out of 138 beds, representing an occupancy rate of 47.8%; 64 patients, 120 accompanying persons and 59 people with disabilities received hot meals.

- **In South Ubangi**, no new admissions or discharges of recovered patients were recorded; 2 patients (1 confirmed and 1 suspected) are in isolation at the Gwaka Ebola Treatment Center (ETC), which has 12 beds, representing an occupancy rate of 16.7%. Five (5) patients have received food assistance since the center opened, with support from the provincial government for nutritional care; the ETC has been equipped with supplies, medications, and glucometers, and the MSF team has assessed the Gwaka Health Center.
- **In Tshopo**, two new admissions were recorded against two discharges. Fifteen (15) patients (6 confirmed PPL) are in isolation for 31 beds, representing an occupancy rate of 48.4%; 8 confirmed, 5 suspected and 3 accompanying persons received a hot meal and 42/42 (100%) frontline providers also received hot meals.
- **In Bas-Uélé**, 4 new admissions of suspected cases, against 3 discharges of non-cases; 6 patients (1 confirmed in Ganga and 5 suspected cases) remain in isolation in the non-standard CTs of Ganga, Poko, Limba and Buta, while the new confirmed case of Viadana has been transferred to the CTE of Isiro.

2.5.2. Challenges

Treatment remains partially substandard and capacity is limited: **In Ituri**, Ebola Treatment Centers (ETCs) are saturated with suspected cases in Nizi, Mambasa, Lolwa, Kilo, Mangala, and Mandima. **In North Kivu**, 31.2% of hospitalized patients are treated outside of standardized facilities; no beds are available in Butembo and Katwa. **In Tshopo**, the lack of safe blood and limited technical resources persist. **In Bas-Uélé**, the absence of ETCs and ambulances hinders the isolation of cases.

2.6. Vaccination

2.6.1. Main actions

- **In Bas-Uélé**, cold chain equipment has been installed at the Buta Expanded Program on Immunization (EPI) center, the ERVEBO vaccine acceptability survey is ongoing among people living with HIV (PLHI) in four health zones, and 99.3% of individual data are entered into DHIS2; **in North Kivu**, cold chain freezers are being delivered, and 36,070 PLHI have been identified in the affected health zones. Tshopo has not yet submitted any vaccination data.
- **In Ituri**, Ebola vaccination continued in the Bunia health zone, where 135 people were vaccinated in the last 24 hours, representing a daily coverage of 45% (135/300). Of those vaccinated, 66 were women (48.9%) and 69 were men (51.1%), including 2 breastfeeding women; no pregnant women were vaccinated today. No adverse events following immunization (AEFIs), severe or non-severe, were reported.
- **In South Ubangi**, deployment of PEP and vaccination experts in the Bulu health zone; calls for emergency vaccination of the community during community dialogue at Gwaka Mission.

2.6.2. Challenges

Coverage of PPLs still incomplete in Ituri (77.0% of the target in Bunia); in Bas-Uélé, ERVEBO stock reported at 0 doses (supply expected); in North Kivu, vaccination of PPLs not yet started; no data from Tshopo.

2.7. Continuity of Care

2.7.1. Main actions

- **In Tshopo**, continued supervision of the affected health zones.

2.7.2. Challenges

Major drug shortages in Boma Mangbetu, Isiro, Wamba and Pawa.

2.8. Risk Communication and Community Engagement (RCCE)

2.8.1. Main actions

- **In Ituri**, 26,382 home visits were conducted, reaching 82,697 people; 1,483 educational talks were held (23,091 people); 179 mass awareness campaigns were organized (12,272 people) and 5 community debates (267 participants); and 9 advocacy campaigns were conducted (54 people); they reported

380 community alerts including 66 deaths, resolved 104 feedbacks out of 140 (74.3%) and 20 incidents out of 28 (71.4%), with a completeness of 32.1% (9/28 ZS).

- **In North Kivu**, CREC teams carried out 10,387 home visits reaching 33,357 people, facilitated 111 educational talks (1,299 people), organized 33 mass communication sessions (1,950 people) and 4 briefings (121 people); they notified 189 out of 450 expected alerts (42.0%), resolved 19 out of 26 feedback within 48 hours (73.1%) and 3 out of 6 incidents (50.0%) and engaged 1,065 out of 4,750 people in community actions (22.4%).
- **In Haut-Uélé**, 421 home visits reached 1,284 people in Isiro and Wamba, 32 educational talks were conducted (356 people) in Pawa and advocacy was carried out at the Governorate (25 people) to strengthen the multisectoral framework and the village approach; they notified 29 alerts of suspected cases and 3 community deaths, resolved 24 feedbacks and supported EDS management and decontamination in Pawa and Wamba.
- **In Bas-Uélé**, 211 home visits reached 755 people, 20 educational talks were held (109 people), 7 mass awareness campaigns were organized (1,928 people) and 5 radio broadcasts were held in the health zones of Viadana, Ganga, Poko, Titule and Buta; they notified and dealt with 6 community alerts, resolved 1 feedback, conducted advocacy with the chief of the Kipate chiefdom (Viadana health zone) and made the new confirmed case and his family aware of the need for his transfer to the Isiro CTE.
- **In South Ubangi**, 374 home visits reached 1,674 people (745 men and 929 women) in the health districts of Gwaka Mission, Mondosa, Kuma, and Akula. A community dialogue was held with 53 people (35 men and 18 women) in Gwaka Mission village, during which the community pledged to cooperate with the response teams and report any suspected cases. Two out of two feedback issues were resolved within 48 hours (rumors attributing all deaths to Ebola and accusing nurses of seeking money), and the Emergency Coordination Zone (ECZ) was briefed on the CREC-SBC (Ebola Response Coordination - Sustainability of the Diseases Center).

2.9. Mental Health and Psychosocial Support (MHPSS)

2.9.1. Main actions

- **North Kivu**, 12 of the 31 confirmed cases (38.7%) from the previous day and 234 confirmed cases under follow-up benefited from SMSPS interventions, as well as 139 new suspected cases and 346 suspected cases under follow-up; 86 households were supported for public health actions and 54 PPLs supported, 7 recovered out of 14 followed in the community (50.0%), the 24 households of separated and orphaned children were supported and 123 results were announced, including 31 positive results; 9 of the 11 health zones that notified are covered and 177 men, 228 women and 99 children benefited from psychoeducation sessions with the support of UNICEF.
- **In Haut-Uélé**, 4 new confirmed cases and 24 confirmed cases under follow-up, as well as 7 new suspected cases and 9 suspected cases under follow-up, benefited from SMSPS interventions; 62 caregivers, 35 PPLs and 33 household members were supported and 19 results were announced, including 9 positive results, with professional reintegration in Boma Mangbetu.

2.9.2. Challenges

- **In Ituri**, low coverage of SMSPS interventions (11 of the 28 health zones covered; 38.7% of the latest confirmed cases); in North Kivu, absence of nurseries in several affected health zones and insufficient human and logistical resources; in Haut-Uélé, lack of computer equipment, difficulty in the mobility of health workers in Wamba and Boma Mangbetu, insufficient nutritional kits for weaned infants and poor data reporting in Boma Mangbetu and Pawa.

2.10. Logistics

2.10.1. Main actions

- In South Ubangi, start of Gwaka CTE site closure.
- Support for the mobility of response teams in the affected provinces; in North Kivu, continuation of construction work at the Musienene CTE and start of construction of cabins at the Mususa PoC; in Bas-Uélé, deployment of the surveillance team to Ganga, continuation of the construction of the transit center at the Buta HGR and rehabilitation of the meeting room and offices of the DPS with the support of the WHO, in a context of limited stock of chlorine (25 kg) and fuel (170 litres); maintenance of vehicles and support for the deployment of inputs for the different pillars.

2.10.2. Security

2.10.3. Main actions

- An EDS team was attacked at the Paidia cemetery (Beni Health Zone) during the burial of a confirmed case, resulting in warning shots being fired, the vehicle being stoned, a death among the population, and injuries among the security agents; EDS and IPC teams were fully supported in the Beni and Katwa sub-coordinations for the burial of 4 confirmed cases, two community death swabs, and the decontamination of 4 households and the Saint-Jean morgue; Support was provided to recover a confirmed case at the Nyankunde Soccer Hospital, who was released with the complicity of the medical staff.

2.11. PSEA (Prevention of Sexual Exploitation and Abuse)

2.11.1. Main actions

- In Ituri, 18 journalists (9 men and 9 women) in Bunia and 67 actors in the response (45 men and 22 women) in Komanda were briefed on the PSEAH; 947 students and teachers were sensitized in Aru (EP Edap) and in Bambu (EP Kilomoto); a preparatory meeting for the assessment of EAS risks was held in Nizi; Haut-Uélé: 512 students and teachers from the Saint-Vincent de Paul college (Isiro), 3 journalists from the ACP, 5 teachers from the EP Ekpoka (Boma Mangbetu) and 31 students in Wamba were sensitized, with 25 codes of good conduct signed.

2.11.2. Challenges

- Still insufficient coverage of PSEA activities, with only 41 of the 119 health zones monitored (34.5%) aware of the reporting mechanism; lack of mapping of stakeholders and refusal of alignment by some partners; insufficient mass awareness materials and lack of means of transport and computer kits for the pillar.

MESSAGES CLES

- Active and persistent community transmission has been observed in the provinces of Ituri, North Kivu, Haut-Uélé, Tshopo, and Bas-Uélé. Meanwhile, the province of Sud-Ubangi has not reported any confirmed cases in the last 24 hours.
- Progress that needs to be consolidated has been made in the area of Epidemiological investigations: almost all (96.7 %) of suspected cases were investigated within 24 hours of notification. However, the contact tracing rate is down compared to the previous day (79.2%).
- Strengthening priority interventions (surveillance, care, IPC, logistics and community engagement) remains essential to rapidly interrupt transmission chains: Rapidly increase reception capacities in ETC/CT in North Kivu and strengthen cross-border surveillance with South Sudan.
- Strengthen preparedness and response to Ebola Virus Disease-B in the provinces of the River Corridor and surrounding areas.

TIMELINE OF KEY EVENTS

- From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.
- Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of funds.



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Director General of INSP

Dr. MWAMBA KAZADI Dieudonné

Email: dieudonne.mwamba@insp.cd

Incident Manager

Professor Dr. AHUKA MUNDEKE Steve

Email: steve.ahuka@unikin.ac.cd

Operations Manager

Dr. KOBBE NGWAMA John

Editorial Team

Professor Dr. MULANGU KATULUMBA Félix-Médard

Dr. KATUSI BISELENGE Jules

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Contact :

www.insp.cd

contact@sgiebola17.org

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